Leuprorelin 22.5 mg: hormone injection every 12 weeks

National Cancer Control Programme (NCCP) 00479 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.

STEP 2

Taking or receiving treatment

A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

STEP 4

Specific instructions

Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.

STEP 5

Leaving with a clear plan

Confirm the exact next dose or appointment, duration and verified daytime and out-of-hours contacts.

Sources: NCCP 00479 v3: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.